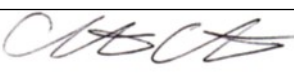


Guideline Title: Diabetic Ketoacidosis (Adult)

Guideline Number: GUID-3203-M002

Issue date: 09/09/2014	Replaces Dept. Policy:
Revision dates: 4/30/24	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** To recognize the patient with diabetic ketoacidosis (DKA) and to maintain adequate oxygenation/ventilation/perfusion, replace deficits, and correct acidosis without causing cerebral edema or severe electrolyte imbalances. Provide expeditious transport to the nearest appropriate medical facility.
- II. **DEFINITIONS:**
 - a. DKA – Diabetic Ketoacidosis
- III. **DEPARTMENT GUIDELINE:**
 - a. Establish care as defined by GUID3203-G002 – *General Patient Care*.
 - b. Assess fluid replacement need. Patients with DKA have an average of 5-7 L of fluid deficit.
 - i. Initially patients should receive a 1000 ml 0.9% NS bolus over 1 hour followed by 500 mL of 0.9% NS per hour for the next 3-4 hours.
 - ii. When plasma glucose reaches 300 mg/dl, fluid should be changed to D5NS with continued insulin therapy.
 - c. Evaluate the need for potassium replacement therapy prior to departing the referring facility. Normal potassium level in an acidotic patient will require potassium replacement as the acidosis is corrected. See GUID3203-M006 *Electrolyte Disorders - Potassium Imbalance*.
 - d. For patients with decreased cardiac contractility and vasodilation that may impair tissue perfusion with a pH <6.9, consider **Sodium Bicarbonate infusion 50 mEq in 250 ml D5W over 1 hour**.
 - e. If fluid resuscitation completed, discuss regular insulin administration with sending or receiving physician.
 - i. Consider **Regular Insulin (100 units/100ml NS) infusion at 0.1 units/kg/hr**.
 - ii. Check a blood glucose level every 30 minutes if patient is receiving insulin therapy.
 - iii. The insulin drip should be continued at the current rate until ketonemia and acidosis have been corrected.
- IV. **DOCUMENTATION:**
 - a. EMS Charts
- V. **REFERENCES:**